



Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD), Inpatient wards/ units	Medical, Nursing, Allied Health, External Security staff and EMHS security services

Purpose

This standard operational procedure (SOP) provides guidance for preventing, reducing, and managing the risk of patient's aggressive or violent behaviours, regardless of whether the behaviour is due to mental illness.

Links within document

- [Appendix I: Communication and de-escalation framework for an aggressive or agitated patient](#)

Early recognition, anticipating and responding to a patient's potential for increased risk of aggressive behaviour in a timely manner has been shown to reduce incidences of aggression.

Procedure details

Considerations

Clinicians are to consider the following principles of care and strategies for all patients in a graded, step-by-step manner if and when behaviour representing agitation or aggression happens.

- Use a multidisciplinary approach, ensure care and management of risk is aligned with the [Chief Psychiatrist's Standards for Clinical Care](#) and **WA Department of Health Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour**
- Consider ethical principles to do or cause no harm; treat people fairly and justly and promote patient's best interests
- Strengthen gender sensitive practice and cultural approaches for culturally and linguistically diverse (CaLD) groups, Aboriginal people, others and the carer, personal support person (PSP) or guardian, if appropriate
- Consider staff emotional regulation and self-management
- Model therapeutic behaviour
- Consider role of leadership by senior clinicians of the multi-disciplinary team (MDT), both during clinical situations and ensuring general awareness of de-escalation and non-pharmacological approaches by staff

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Version: 1.1

Endorsed by: Clinical Practice Committee 07/12/2023

Authorised by: No Patient Harm Committee 23/01/2024

DMR Amendment: Clinical Practice Committee Chair OOS 28/03/2025

Date Compiled:

08/2019

Executive Sponsor:

Co-Director MH Division

Revision Due:

01/2027

Procedure details – Considerations cont'd

- Proactively identify and manage incidents of teasing, bullying, unwanted physical or sexual contact or miscommunication between patients
- Promote social interaction, redirection, distraction techniques and reducing boredom, e.g., pet or music activities, dance interventions, exercise groups or programs on units, use of comfort rooms, sacred spaces, etc. Refer to **Comfort Room Use SOP (Mental Health)**
- Services should ideally attempt to develop consensus as to what constitutes aggressive behaviour requiring interventions specific to the area's environment. Whilst it is impossible to have total agreement, this process should help develop consistency in responding to any clinical situation
- Develop clarity regarding roles between nursing/treating team and security officer teams (Refer to **EMHS Security Policy** and/or the **RFBG Security: Request for External Security Staff SOP**), as applicable.

Assessment

- Early assessment to identify the following
 - current presentation: reason and precipitating events and/or factors
 - comprehensive history of aggressive behaviours and forensic history
 - any outstanding legal matters
 - substance and/or alcohol misuse: recent and historical
 - triggers for escalation: environmental, personal, and staffing
 - previously known effective de-escalation techniques
- Refer to **Recognising and Responding to Acute Deterioration (RRAD) in Mental State, Cognition and Behaviour SOP** and **RRAD Policy**
- Review alerts on Psychiatric Services Online Information System (PSOLIS)
 - Identify the risk of aggression and the form in which it commonly presents such as verbal or physical – consider previous or update Risk Assessment and Management Plan (RAMP) and/or Progressive Risk Assessment (PRA) (if clinically indicated)
 - Obtain information from patient, referrer and carer, PSP, or guardian
 - Refer to or complete the patient's Treatment Support, Discharge Plan (TSDP), Comprehensive Care Plan, What Matters Most Record (WMMR) or other care plans, factoring in the patient's preferences to reduce distress and agitation
 - Consider referral to specified assessment and interventions where appropriate (e.g., Acute Pain Service) and/or other referrals, e.g., external community services
 - Refer to **Potentially Harmful Items Management for Mental Health Patients SOP**

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Procedure details cont'd

Engagement with patient

- Proactively build patient rapport early with consistency of staff
- Before entering a **patient's bed space or environment**, staff should consider if they can reduce risk to themselves by:
 - Removing lanyards, stethoscopes, or other ligatures from around their necks (all lanyards must have at least two break points)
 - Removing pens, scissors from accessible pockets
 - Tying up hair, removing any hoops or long earrings.
- On entering the patient environment, initiate communication with the patient at a safe distance and:
 - On approaching the patient, assess the patient's verbal and non-verbal language, and undertake a dynamic risk assessment to determine changes in patient risk status
 - If safe, proceed with caution maintaining situational awareness
 - Position self near the exit and ensure easy access to personal alarms (if appropriate to area)
 - If the staff member feels unsafe – to remove themselves and seek further support
 - Use trauma informed care principles to assist the patient to understand and develop positive coping strategies (e.g., self-modulation, self-regulation, prompting [can be done verbally or non-verbally])
- Acknowledge the patient's right to feel angry or frustrated and support their use of identified coping strategies
- When de-escalating, use a variety of verbal and non-verbal skills and interactional techniques to avoid and manage known 'flashpoint' situations as appropriate to the situation (refer to Appendix I: Communication or De-escalation Framework)
- Involve carers, PSPs, or guardians, where possible and offer peer coaching (if available).

De-escalation and management of patient aggressive behaviour

Throughout admission, ongoing review of:

- Purpose for admission
- Mental state
- Discharge planning
- Any recent or ongoing stressors
- Known triggers and de-escalation factors
- Medication: regular and prn
- Sleep, appetite, and physical health issues
- Access to quiet and/or exercise areas
- Psychosocial support for family and/or support systems
- Coping strategies
- Alcohol or substance misuse.

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Procedure details cont'd

Management of escalation of patient risk

Senior Clinicians are to

- Assess the patient and determine interventions as appropriate in a stepwise manner and any interim actions (including medications, type and frequency of observations, requirement for special and/or security)
- Ensure changes are reported and discussed with MDT, during clinical handover and safety huddles for appropriate review
- Consider referral to specified assessment and interventions where appropriate (e.g., Acute Pain Service) and/or other referrals, e.g., external community services
- Consider liaison with Safety All Hours for Everyone Team (SAFE) Royal Perth Hospital (RPH), Bentley Health Service (BHS) - BeSAFE, or After Hours Mental Health (MH) Clinical Nurse Specialist (CNS) to identify Patient of Concern
- Commence or review required documentation, e.g., RAMP/PRA, WMMR and TSDP
- Update a PSOLIS risk alert and/or on Enterprise Bed Management (EBM) and on iClinical Manager (electronic handover)
- Complete mental health review at the earliest possible
- Request/complete second opinion if required and/or indicated
- Review for MHA 2014 applications and requirements
- Activate a Code Black if indicated
 - Document activation and subsequent review in the patient's healthcare record
- If attempts at de-escalation and other interventions are proving ineffectual or not able to be initiated or causing imminent risk of serious harm to patients, staff or others or peril to life, consider restrictive practices as measures of last resort (Refer to **Restraint and Seclusion Minimisation and Safe Management [for Mental Health Areas] CPS**)
- Seek further guidance relating to escalation of patient care, including:
 - **RRAD Policy**
 - **RRAD in Mental State, Cognition and Behaviour SOP**
 - **Escalation to Consultants Policy**
 - **SAFE and BeSAFE SOP**
 - **WA Department of Health Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour**

Post-incident patient or staff support and documentation

For patient support and/or PSPs or guardians

- Follow the **Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP**
- Review or update the patient's PRA or RAMP, comprehensive care plan, TSDP, WMMR or other care plans and discuss with the patient their triggers to the event and explore how staff may be able to assist in minimising these triggers
 - Ensure patient's approval and signature of the updated patient's TSDP and WMMR, and provide copies to patient and carer, PSP, or guardian, as consented.

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Procedure details – Post-incident patient or staff support and documentation cont'd

Physical injuries



Perform hand hygiene

- Provide first aid as appropriate
- Consider if medical attention required
- For any biological exposure – i.e., spitting affecting mucous membrane, biting where skin is broken, needle stick or other, follow procedure: **Occupational Exposure to Blood/Body Fluids SOP**.

For staff support

- Refer to the Employee Assistance Program (EAP), RPBG Centre for Wellbeing and Sustainable Practice and the **EMHS Post Incident Management / Critical Incident Stress Debriefing Policy**
- Access to regular clinical supervision
- Offer individual support or debriefing where appropriate
- Refer to the [EMHS Work Health and Safety \(WHS\)](#) and [Stop the Violence](#) HealthPoint hub pages.

Reporting incidents

- In the event of staff assault, Ward or Unit Manager or Leader to discuss process for reporting to police. Provide FAQ: Reporting an Incident of Workplace Aggression or Violence to the WA Police or refer to the [Stop the Violence](#) hub page on HealthPoint
- Complete a PSOLIS risk alert, on EBM and on iClinical Manager – electronic handover
- Complete an incident report via the Datix Clinical Incident Management System (CIMS) or Combined Hazard and or Incident Report System (CHoIR) notifications or Office of the Chief Psychiatrist (OCP) Notifiable Incident reporting.

Training

- All clinicians are to receive training on de-escalation techniques and API as per the **EMHS Mandatory Training Policy**
- EMHS will deliver training on the [Workplace Inspection Checklist](#) and [Environmental Aggression Risk Assessment \(EARA\) Tool](#) to Work Health and Safety Representatives working in MH Inpatient areas.

Compliance monitoring

Compliance with this SOP will be monitored via the following:

- Datix CIMS or CHoIR notifications or OCP Notifiable incident reporting
- Compliments and Complaints
- De-escalation training compliance rates will be monitored by Line Managers.

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We acknowledge the following previous site endorsed work and/or contributors used to compile this document.

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP through consultation with EMHS Aboriginal Health Strategy Team, Aboriginal health Liaison Officers and Aboriginal Acute Care Coordination Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3017).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

[RPBG Policy Hub](#)

- [Comprehensive Care SOP](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Comfort Room Use SOP \(Mental Health\)](#)
- [RRAD in Mental State, Cognition and Behaviour SOP](#)
- [RRAD Policy](#)
- [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- [Restraint and Seclusion Minimisation and Safe Management \(for Mental Health Areas\) CPS](#)
- [Escalation to Consultants Policy](#)
- [SAFE and BeSAFE SOP](#)
- [Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP](#)
- [Occupational Exposure to Blood/Body Fluids SOP](#)
- [Security: Request for External Security Staff SOP](#)
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Related policies, practice standards, clinical guidelines cont'd

EMHS Policy Hub

- [Duress Alarms Systems Policy](#)
- [Rights of Patients in Mental Health Policy](#)
- [Post Incident Management / Critical Incident Stress Debriefing Policy](#)
- [Mandatory Training Policy](#)
- [Security Policy](#)

WA Department of Health Policy Hub

- [SSCD for Mental Health Services Procedure](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- [Charter of Mental Health Care Principles \(MHA 2014\) – WA Government](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist – Public Mental Health 2018 - Office the Chief Psychiatrist](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Mental Health Act 2014](#)

Related forms

- *Risk Assessment Management Plan (RAMP)*
- *Agitation and Arousal PRN Medication Chart*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 1: Clinical Governance

Standard 2: Partnering with Consumers

Standard 5: Comprehensive Care

Standard 6: Communicating for Safety

Standard 8: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010)

Standard 1: Rights and responsibilities

Standard 2: Safety

Standard 10: Delivery of care.

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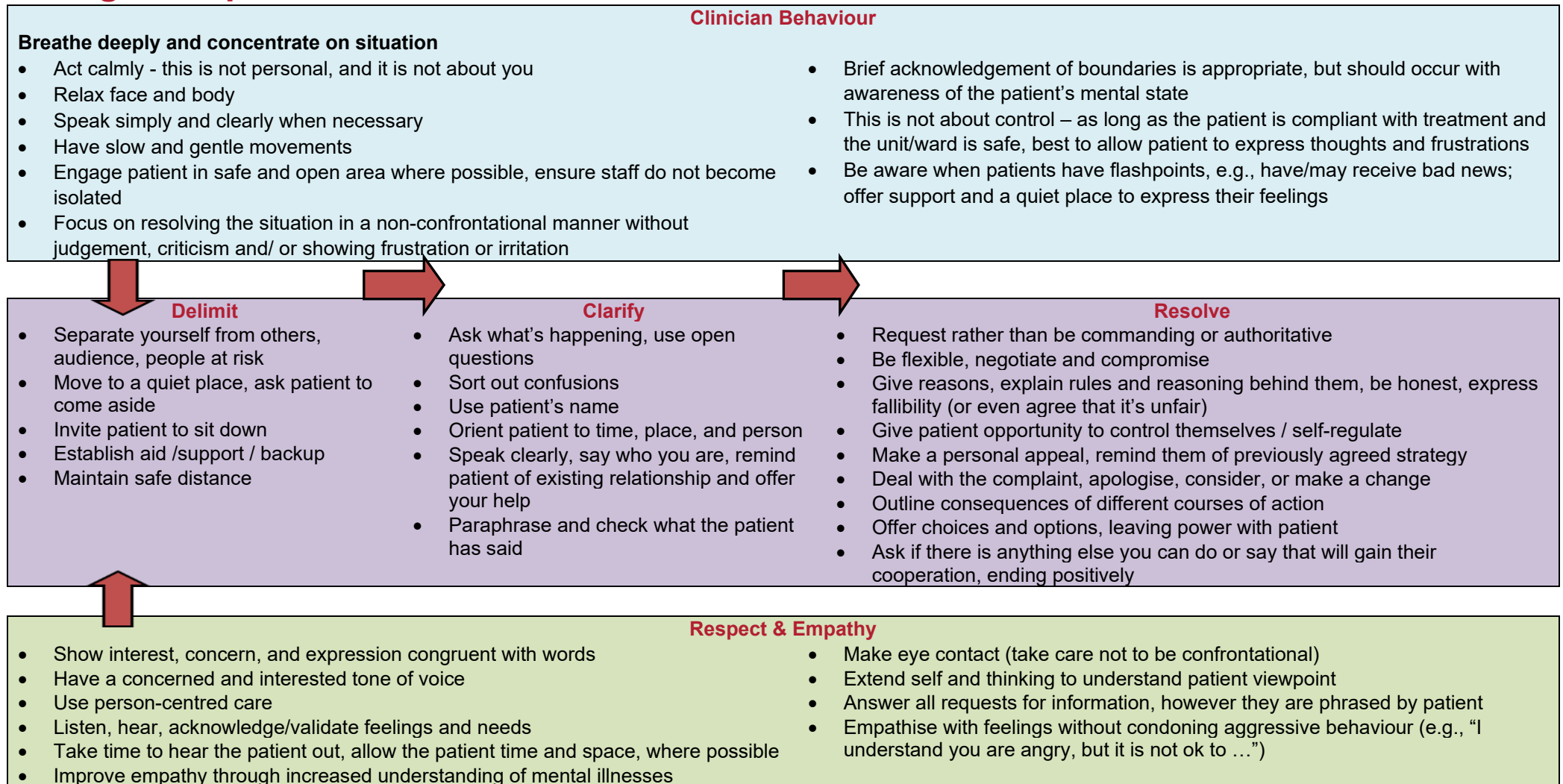
Revision Due:

08/2019

Co-Director MH Division

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Appendix I: Communication and de-escalation framework for an aggressive or agitated patient



Acknowledgement: Adapted from Safewards. 2019. URL: <http://www.safewards.net/>

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08/2019

Document Owner:

MH Division Co-Director

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Authorised by:

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Revision Due:

01/2027

DMR Amendment Clinical Practice Committee Chair OOS 28/03/2025